

ختم
شعار الجمهورية

Name: منال حنا جاب الله رزق الله
National ID: 27609052402006



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Pin Medication And Anti Emetics

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

اشفاعة

جمهورية مصر العربية
بطاقة تحقيق الشخصية

مقال
حناء جاب الله رزق الله
٨ حوض الیومات - الكوم الأخضر
الهرم - الجيزة

١٩٧٦/٠٩/٢٥ ٢٧٦.٩.٥٢٤.٢٠٠٦
JE2719286

~~١٩٨٨/٩/٢٧~~

١ ms ~~موسى حنا موسى حنا~~

مسح ذری
على السقام

عاجل
جيد

٢٠٢١/١٢ ٢٧٦.٩.٥٢٤.٢٠٠٦
ربة منزل

أنشى مسيحية متزوجة
موسى حنا موسى حنا
البطاقة مصرية حتى ٢٠٢٨/١٢/٢١

١



NHTMRI

قسم الباثولوجي

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: منال حنا جاب الله رزق

Request type: مبادرة

Hospital no:

Department: صحة مرأه

Referred by: د/رحاب الشيخ

Nature of specimen: tru-cut biopsy

Clinical diagnosis: Left breast mass

Age: 59years

Sex: female

Sub request type:

Pathology no.:468B-2026

Date of result:11/6/2026

Date of biopsy:4/6/2026

I.D. Number:26709052402006

Gross pathology :

Multiple grayish white tissue cores 0.5x0.5cm, totally submitted.

Immunohistochemical technique: Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: Ecadherine -ER- PR- Her2- KI67- Pan CK

Microscopic picture:

Microscopic examination revealed mammary core tissue showed malignant growth formed of tubules, clusters, nest, cribriform pattern and single anaplastic cells with marked anaplasia and mild poor tendency to form tubules dispersed among desmoplastic stroma with mild lymphocytic response.

Immunohistochemistry:

ER: Weak nuclear staining in < 1 % of tumor cells (Allred Score 2/8) Negative.

PR: Weak nuclear staining in < 1 % of tumor cells (Allred Score 2/8) Negative.

Her2: Strong complete membranous staining of >10% tumor cells (score 3+) Positive

Ki67: Positive nuclear staining of 70% of tumor cells.

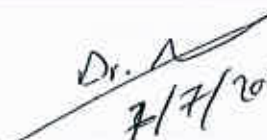
Diagnosis: Left breast mass Tru-Cut Biopsy:

Invasive mammary carcinoma, Grade III

Signature:

Dr.Nermine Nabil.M.D

Dr.Mona.Mohamed.M.D

Dr. 
7/7/2026

Pathology Lab. Manager

BASMA REFAAT M.D

Patient ID: 95984	Patient Name English: Manal Hana Gab Allah Rezk
Age: ٤٩ سنة ٨ شهر	2026/06/04 11:42:04 AM :Report Date

Bilateral Full field Digital Mammography with complementary ultrasound

Clinical summary: 45 Years old female with left breast palpable lump

Technique: Medio-lateral oblique, CC views and High resolution ultrasound

Mammographic Findings

- Heterogeneous dense breast parenchyma (ACR-c)
- **Left breast:**
 - UOQ anterior, middle and posterior thirds irregular shaped spiculated margin heterogeneous dense lesion measuring 4.6 x 4.4 cm and about 7.2 cm distance from the nipple
 - Global asymmetry and parenchymal distortion
 - Pleomorphic microcalcifications in segmental distribution measuring 11 x 7 cm
 - The overall pathologic area measuring 13.8 x 13 cm
 - Moderate diffuse skin thickening and trabecular thickening
 - The pathological lesion reaching the nipple with subsequent slight nipple retraction
 - The axilla in the MLO view showing about two irregular shaped spiculated margin lesions?? lymph nodes with extra-capsular extension measuring about 12 x 10 mm and 16 x 13 mm from superior to inferior respectively
- **Right breast**
 - No gross evidence of dense spiculated lesions or parenchymal distortion
 - No suspicious microcalcifications noted
 - Normal skin thickness and contour. Normal nipple / areola complex.

Ultrasound findings:

- Non-homogeneous breast parenchyma- predominant glandular
- **Left breast:**
 - Upper outer quadrant 1-3 o'clock diffuse pathologic process evident as multiple irregular shaped heterogeneous hypoechoic solid lesions with indistinct margins, the parenchyma in-between the lesions showing altered echogenicity (heterogeneous hypoechoic)
 - The largest is 2-3 o'clock measuring 3 x 2.5 x 2.4 cm and 1.5 cm distance from skin
 - At 2 o'clock another larger lesion measuring 2.6 x 1.6 cm reaching the skin
 - Moderate diffuse skin thickening up to 7 mm and mild to moderate interstitial edema
 - **Regarding the axillary lymph nodes**
 - o More than 5 level I axillary lymph nodes with globular shape, diffuse cortical thickness and effaced hila
 - o Two left axillar tail irregular shaped lesions with indistinct margins, one of them showing echogenic focus?? Calcifications (corresponding to the axillary tail spiculated margin lesions at MLO view)?? Could be representing extracapsular extension?? rather than another satellite lesions; the largest measuring 11 x 10 mm
 - o Less than three level II axillary lymph nodes

9/13/26, 9:46 AM

code:09 – Name_A MDT FORM – Name_E MDT FORM

جوسناف
روبياني
مستشفى جامعة القاهرة

أ.د. هاني
9-12



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية
مصرية
الجديدة

09/13/2026

MDT FORM

Name:	Manal عبد الله رزق	MRN:	12277
Age:	50 Y	ID:	27609052402006
Menopausal Sta	post	Phone:*	01288192786
NRS SCORE:		Marital Status:	
BMI:		Family History:	-ve fh
Gender		Reason Of MDT:	new case
CO Morbidity:	m.free	ECOG Performance Status	0 00 10 20 30 4
Surgical History:*	FREE		

Clinical Data:

BX: It. Invasive mammary carcinoma G3

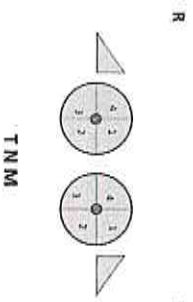
ER: 2/8 **PR:** 2/8 **HER2:** 3+ **KI67:** 70%

History:

Sonomammography:

ACR C

LT. UOQ MASS 4.6*4.4 7.2 CM FROM NIPPLE
POLEOMPHORHIC MICROCALCIFICATION 11*7 CM
DIFFUSE SKIN THICKENING LESION REACHING NIPPLE WITH SLIGHT
NIPPLE RETRACTION
AXILLA MORE THAN 5 LEVEL 1 & 2 TWO LEFT AXILLARY TAIL LESIONS
LT SUPRA CLAVICULAR LN 2.4*1.3 CM BIRADS 5
RT BREAST SINGLE LEVEL 1 INDETERMINATE AXILLARY LN 0.32 CM ??
BIRAD 3
CESM
LT UOQAT 2 OCLOCK MASS WITH ARCHITECTURAL DISTORTION AND
MICROCALCIFICATION WITH RETROAREOLAR EXTENSION 11*4.5*14 CM
ANOTHER LESION AT AXILLARY TAIL 2.3*2.0 CM
DIFFUSE SKIN THICKENING
MULTIPLE AXILLARY LN LEVEL 1 & 2 BIRADS 6



T N M

Clinical Staging:

CT4BN3M1

Pathological Staging:

Biomarker/Molecular

Mets Workup:

CT CAP >> FREE
BONE SCAN>>> MULTIFOCAL ACTIVE SKELETAL LESIONS
HIGHLY WIDESPREAD OSTEOBLASTIC METASTATIC BONE DISEASE
SKULL DOME, LOWER THORACIC & LUMBER SPINE ,BL. RIBS RT SCAPUL,
SACRUM

Recommendations:

MDT Outcome & Plan

for taxol /herceptin
zometa every 3 ms after dental assessment

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. هاني عزت	د. أحمد لطفي	د. هاني عزت	د. أحمد لطفي

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مبادرة رئيس الجمهورية
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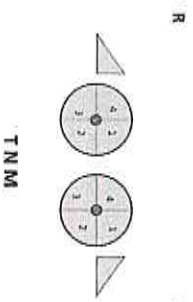
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DIFFUSE SKIN THICKENING

MULTIPLE AXILLARY LN LEVEL 1& 2BIRADS 6



	Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4
Right Side								
Left Side								

Clinical Staging:

CT4BN3M1

Pathological Staging:

Biomarker/Molecular

Mets Workup:

CT CAP >> FREE

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